

FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg)

Product Name

Generic name: Fluticasone Nasal Spray BP 50mcg

Trade name: FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg)

Name and Strength of Active Ingredient (s)

Name: Fluticasone Propionate

Strength: Fluticasone propionate 0.05% w/w (Fluticasone propionate BP 50 mcg per actuation)

Dosage Form

Aqueous suspension for intranasal inhalation via metered-dose valve system

Product Description

FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg) is a white aqueous suspension with aromatic smell of microfine fluticasone propionate (0.05% w/w) for topical administration to the nasal mucosa by means of a metered-dose valve system. Each spray delivered by the nasal adaptor contains fluticasone propionate 50mcg.

FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg) is supplied in a brown glass bottle fitted with a metered-dose valve system, nasal adaptor and a dust cover.

Pharmacodynamics

Fluticasone propionate is a synthetic, trifluorinated corticosteroid, it has local anti-inflammatory and anti-allergic activity.

Pharmacokinetics

Absorption: Direct absorption in the nose is negligible due to the low aqueous solubility with the majority of the dose being eventually swallowed. When administered orally, the systemic exposure is <1% due to poor absorption and presystemic metabolism. The total systemic absorption arising from both nasal and oral absorption of the swallowed dose is therefore negligible.

Distribution: Fluticasone propionate has a large volume of distribution at steady state. Plasma protein-binding is moderately high.

Metabolism: Fluticasone propionate is cleared rapidly from the systemic circulation, principally by hepatic metabolism to an inactive carboxylic acid metabolite, by the cytochrome P-450 enzyme CYP3A4. Swallowed fluticasone propionate is also subject to extensive first-pass metabolism. Care should be taken when co-administering potent CYP3A4 inhibitors eg, ketoconazole and ritonavir, as there is potential for increased systemic exposure to fluticasone propionate.

Elimination: The elimination rate of IV-administered fluticasone propionate is linear over the 250- to 1000-mcg dose range and are characterized by a high plasma clearance. Peak plasma concentrations are reduced by approximately 98% within 3-4 hrs and only low plasma concentrations were associated with the 7.8 hrs terminal half-life. The renal clearance of fluticasone propionate is negligible and <5% as the carboxylic acid metabolite. The major route of elimination is the excretion of fluticasone propionate and its metabolites in the bile.

Indications

The prophylaxis and treatment of seasonal allergic rhinitis (including hay fever) and perennial rhinitis.

Recommended Dose

FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg) is administered by the intranasal route only. Shake gently before use.

Prophylaxis and Treatment of Seasonal Allergic and Perennial Rhinitis:

Adults and Children >12 years: 2 sprays into each nostril once a day, preferably in the morning. In some cases, 2 sprays into each nostril twice daily may be required. The maximum daily dose should not exceed 4 sprays into each nostril.

Children 4-11 years: 1 spray into each nostril once a day, preferably in the morning. In some cases, 1 spray into each nostril twice daily may be required. The maximum daily dose should not exceed 2 sprays into each nostril.

Elderly: The normal adult dosage is applicable.

For full therapeutic benefit, regular usage is essential. The absence of an immediate effect should be explained to the patient as maximum relief may not be obtained until after 3-4 days of treatment.

Route of Administration

FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg) is administered by the intranasal route only.

Contraindication

Patients with hypersensitivity to any of the ingredients of FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg).

Warning and Precautions

Local infections: infections of the nasal airways should be appropriately treated but do not constitute a specific contra-indication to treatment with FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg).

The full benefit of FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg) may not be achieved until treatment has been administered for several days.

Care must be taken while transferring patients from systemic steroid treatment to FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg) if there is any reason to suppose that their adrenal function is impaired.

Although FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg) will control seasonal allergic rhinitis in most cases, an abnormally heavy challenge of summer allergens may in certain instances necessitate appropriate additional therapy.

Systemic effects of nasal corticosteroids may occur particularly at high doses prescribed for prolonged periods. These effects vary between patients and different corticosteroids. These effects are much less likely to occur than with oral corticosteroids and may vary in individual patients and between different corticosteroid preparations. Potential systemic effects may include Cushing's syndrome, Cushingoid features, adrenal suppression, growth retardation in children and adolescents, cataract, glaucoma and

more rarely, a range of psychological or behavioural effects including psychomotor hyperactivity, sleep disorders, anxiety, depression or aggression (particularly in children). Growth retardation has been reported in children receiving some nasal corticosteroids at licensed doses. It is recommended that the height of children receiving prolonged treatment with nasal corticosteroids is regularly monitored. If growth is slowed, therapy should be reviewed with the aim of reducing the dose of nasal corticosteroid, if possible, to the lowest dose at which effective control of symptoms is maintained. In addition, consideration should be given to referring the patient to a paediatric specialist. Treatment with higher than recommended doses of nasal corticosteroids may result in clinically significant adrenal suppression. If there is evidence for higher than recommended doses being used then additional systemic corticosteroid cover should be considered during periods of stress or elective surgery. Ritonavir can greatly increase the concentration of fluticasone propionate in plasma. Therefore, concomitant use should be avoided, unless the potential benefit to the patient outweighs the risk of systemic corticosteroid side effects. There is also an increased risk of systemic side effects when combining fluticasone propionate with other potent CYP3A inhibitors.

Interactions With Other Medicaments

Under normal circumstances, low plasma concentrations of fluticasone propionate are achieved after inhaled dosing, due to extensive first pass metabolism and high systemic clearance mediated by cytochrome P450 3A4 in the gut and liver. Hence, clinically significant drug interactions mediated by fluticasone propionate are unlikely.

Co-treatment with CYP3A inhibitors, including cobicistat-containing products, is expected to increase the risk of systemic side-effects. The combination should be avoided unless the benefit outweighs the increased risk of systemic corticosteroid side-effects, in which case patients should be monitored for systemic corticosteroid side-effects.

Pregnancy and Lactation

Use in pregnancy: There is inadequate evidence of safety in human pregnancy. Administration of corticosteroids to pregnant animals can cause abnormalities of foetal development, including cleft palate and intra-uterine growth retardation. There may therefore be a very small risk of such effects in the human foetus. It should be noted, however, that the foetal changes in animals occur after relatively high systemic exposure; direct intranasal application ensures minimal systemic exposure. As with other drugs the use of FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg) during human pregnancy requires that the possible benefits of the drug be weighed against the possible hazards.

Use in Lactation: The secretion of fluticasone propionate in human breast milk has not been investigated. Subcutaneous administration of fluticasone propionate to lactating laboratory rats produced measurable plasma levels and evidence of fluticasone propionate in the milk. However, following intranasal administration to primates, no drug was detected in the plasma, and it is therefore unlikely that the drug would be detectable in milk. When FLUTIMATE NS 50 (Fluticasone Nasal Spray BP 50mcg) is used in breast feeding mothers the therapeutic benefits must be weighed against the potential hazards to mother and baby.

Side Effects

Adverse events are listed below by system organ class and frequency.

Immune System Disorders: Very Rare: Hypersensitivity reactions, anaphylaxis/anaphylactic reactions, bronchospasm, skin rash, oedema of the face or tongue.

Nervous System Disorders: Common: Headache, unpleasant taste and smell. As with other nasal sprays, unpleasant taste and smell, and headache have been reported.

Eye Disorders: Very Rare: Glaucoma, raised intraocular pressure, cataract.

A very small number of spontaneous reports have been identified following prolonged treatment. Intranasal fluticasone propionate have not been shown to be associated with an increased incidence of ocular events including cataract, increased intraocular pressure or glaucoma.

Respiratory, Thoracic and Mediastinal Disorders: Very Common: Epistaxis. Common: Nasal dryness, nasal irritation, throat dryness, throat irritation. Very Rare: Nasal septal perforation.

As with other nasal sprays, dryness and irritation of the nose and throat, and epistaxis have been reported. Nasal septal perforation has also been reported following the use of intranasal corticosteroids.

Symptoms and Treatment of Overdose

Administration of doses higher than those recommended over a long period of time may lead to temporary suppression of adrenal function. In these patients, treatment with fluticasone propionate should be continued at a dose sufficient to control symptoms; adrenal function will recover in a few days and can be monitored by measuring plasma cortisol.

Storage Condition : Store below 30°C. Protect from frost and light.

Shelf Life : 36 months.

Dosage forms and packaging available : 1 unit per box

Manufacturer

Jewim Pharmaceutical (Shandong) Co., Ltd.

Tai'an High-Tech Industrial Development Zone, Shandong Province, PRC

HOW TO USE YOUR SPRAY PROPERLY



- 1). Shake the bottle and take off the dust cap.
- 2). Blow your nose gently.
- 3). Close one nostril with your finger, and put the nozzle in the other nostril. Tilt your head forward slightly and keep the bottle upright. Hold the bottle as shown in diagram 1.
- 4). Start to breathe in slowly through your nose. While you are breathing in press down firmly on the collar with your fingers as shown in diagram 2. A spray of fine mist will go into your nostril.
- 5). Breathe out through your mouth.
- 6). Repeat step 4 to use a second spray in the same nostril.
- 7). Remove the nozzle from this nostril and breathe out through your mouth.
- 8). Repeat steps 3 to 6 for your other nostril.
- 9). After using your spray, wipe the nozzle carefully with a clean tissue or handkerchief, and replace the dust cap.

Date of revision

06 February 2018